



AGM Agenda July 2026

- Welcome and opening statement
- Approval of previous AGM minutes - attached
- CIO
- New board members

Nominations, voting

- Education Report and Standards Report
- Treasurers Report - attached
- Membership Report - attached
- Barna Strategy 2026 - 2027
- Open forum to members
- Closing Statement



Annual Report 2026

- **Charity Commission**

BARNA has converted fully to Charity Commission registration , actioned by Patricia Smedley, former chair.

- **Education Report and Standards Report**

Barna has developed an Education Statement which is designed to set training and education standards. It is also a tool to support educators and trainees in their roles and professional progression within PACU.

In developing the Adult Competency document as, an organisation supporting PACU practitioners, we felt it was important to launch the document to support educators to maintain a safe standard of care that has a national standardisation.

The Paediatric PACU competency workbook was also launched in 2026 after a collaboration with other national organisations endorsements.

Statement on Corridor Care. This document was produced with a multidisciplinary collaboration.

Following the leadership round table listening event, BARNA released workforce standards for clarification and guidance of staffing levels in the Post Anaesthetic Care Unit.

A rolling programme of webinars was initiated in 2025. The use of Eventbrite booking system works well as a whole. The inclusion of company's representation has enhanced the programme. Attendance numbers have steadily increased over the year.

- **Treasurers Report**

Auditors/ accountants – no change from previous.

Our current accountant remains Rod George Ltd. Chartered Accountant whose offices are at 18 High Street. Collingham. NG23 7LA

Handover to new treasurer complete – Emma Mathews

Vote on assistant treasurer role – nominations

Vote on retention of Auditor

- **Conference 2026**

We booked a different venue, KCL site, Bush House, following delegate feedback from the 2025 conference. Delegate attending numbered 113 which was slightly less than previous years. Possible reasons potentially finance. There was a lack of international applicants. There was a significantly higher number of delegates via organisational funding in comparison with 2025.

Feedback for the 2026 conference was overall positive for venue and speakers - attached.

- **International Affiliations**

Our collaboration with IFNA and ICPAN continues with representation from a BARNA Board Members on the executive council for this organisation.

IFNA report from Manda Dunne – attached – from the World Congress of Nurse Anaesthetists 2026

- **Membership Report**

We have engaged with members by listening to our members through feedback and quarterly communications via the new BARNA newsletter.

- **Barna Strategy 2026 – 2027**

Treasurers Report - 11th July 2026 -Emma Matthews- BARNA Treasurer

Cash in Bank (rounded)

Barclays	£ 21,863.00
PayPal	£ 2,340.00

Sub Tot	£ 24,203.00

Conference Related Income

Eventbrite

Ticket income £5,890.00

Invoiced income

Paid Invoices

Sponsors £3,000.00

Delegates £1,200.00

Invoices outstanding

Sponsors £2,250.00 (3 sponsors)

Delegates £2,880.00 (24 delegates)

Total £9,080.00 (34 delegates + 7
sponsors)

Conference related expenses

Hotels and meal for speakers £759.09

Supplies £853.62

Speaker gifts £221.05

Total expected balance £29,333

Plan

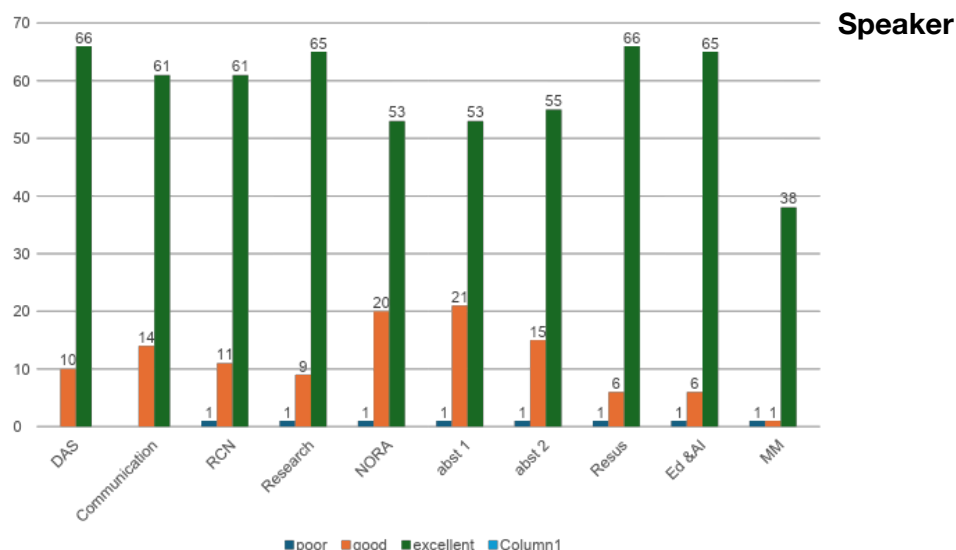
- open Savings bank account to gain more interest on money sitting in balance
- Paypal charge us a fee for every payment received. Membership £50, we only see £48.25 ?changing payment options

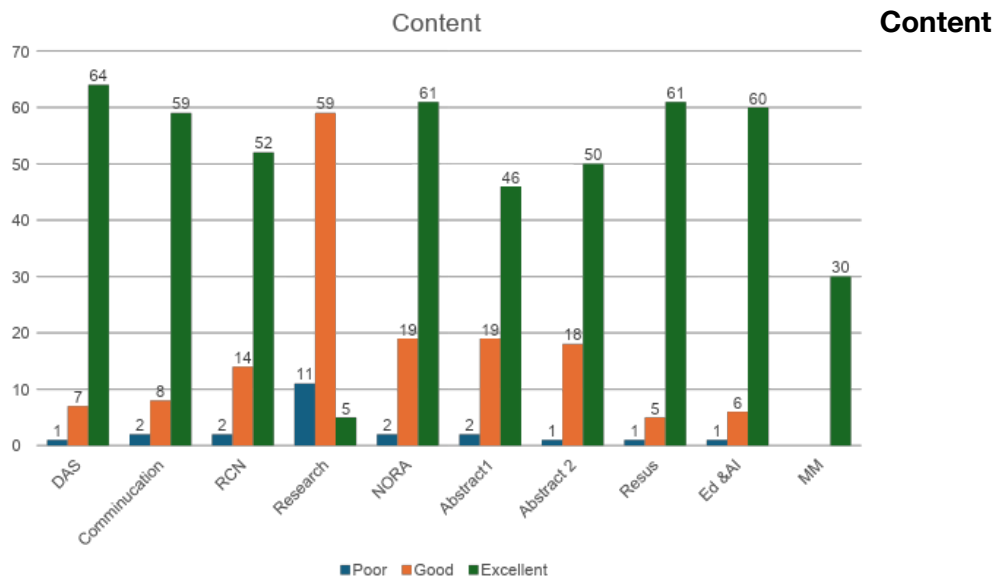
36TH Barna Conference Feedback

Delegate Feedback – 113 delegates, 78 responded = 88% response

Majority feedback was positive with a score of predominately excellent and good for speakers and content.

Venue feedback was also generally good to excellent, however, some comments regarding lack of food provision during morning coffee, seating during lunch, and availability of hot water. This has been fed back to the venue catering.





WCNA 2026 BRISBANE AUSTRALIA

Welcome to Brisbane, Australia for the 15th World Congress of Nurse Anaesthetists 2026. The congress was organised and hosted by the Australian College of Peri Anaesthesia Nurses, on behalf of the IFNA (International Federation of Nurse Anaesthetists). BARNA has been a member of the IFNA for many years and this was the first time the WCNA was held outside of Europe,

indeed, on the other side of the world! BARNA hosted the congress in Glasgow in 2016 on behalf of IFNA, maybe some of you were there!

I can safely say that I have never travelled so far in my life or been on a plane for so long, but my goodness, it was worth it! I believe I flew some 10,000 miles or 16,000 km which for me, is quite an achievement but also the most magnificent opportunity and privilege to represent you all.

I learned that Brisbane is a very relaxed state, and that I was soon saying G'day and what are we doing in the arvo (afternoon!)



What an honour to represent the United Kingdom as

the IFNA (International Federation of Nurse Anaesthetists) Country Representative for the United Kingdom at the World Congress of Nurse Anaesthetists (WCNA) 2026.

I have been the CNR (Council of National Representatives) for many years but never imagined I would one day be waving the flag on behalf of BARNA in Brisbane, Australia.

This was truly an honour and I was there for every anaesthetic and recovery nurse in the UK and for everyone who cares for our patients before, during and after anaesthesia.

The atmosphere was electric and I was filled with pride and excitement! The opening ceremony never fails to give me goosebumps!

Of course no trip to Australia would be complete without these guys! It was lovely to meet them!



The congress began with an Aboriginal Blessing sung beautifully by a Local First Nations Representative, followed by the Governor of Queensland who gave a warm and meaningful welcome. Vera Meeusen, the Congress Chair then welcomed us and the flag ceremony began. It's quite a sight to see all 49 flags of the country members of IFNA paraded across the stage. It brings to life the scale of the IFNA membership and indeed how it has grown even in my time with the Federation. Please remember, your membership to BARNA includes membership to IFNA too.

Pascal Rod, Executive Director, and Jackie Rowles, IFNA President gave their reports. (as a note, following the congress Jackie is now the Executive Director as Pascal Rod has taken retirement). After a very long, dedicated and faithful service to IFNA as Executive Director, Pascal gave a meaningful and heartfelt address where you could feel his passion and love for IFNA to whom he gave thirty eight years amazing service.

This is an eye opening session which highlights what IFNA is involved in and how much work is undertaken by the board and committees to enable recognition, education and support to nurse anaesthetists across the globe. We also heard about their APAP (Approval Process for Anaesthesia Programmes) and where in the world these programmes have been registered, recognised or approved.

Did you know, Nurse Anaesthetists have been in existence for 170 years, in the late 1800's Catholic Nuns trained nurses in many countries including Brundi, Serbia, Switzerland and the US to administer anaesthesia. Physicians often trained nurses outside of religious orders too. Nurses routinely administered anaesthesia to military personnel on the front lines including civil wars, world wars and global conflicts serving in areas of combat, on navy ships, aircraft and global evacuation teams. This included nurses from the UK, who were on the front lines in both WW1 and WW2 delivering and assisting in anaesthesia.

Nurse anaesthetist/anaesthetic nursing practice may vary across the globe but the common denominator is that we are all nurses caring for patients undergoing anaesthesia, and carry a specialised training in order to do so.

The keynote speech was delivered by Dr. Jonathan Chejfec and he spoke about 'When Systems Fail'. These were the lessons learned from the Mexican Fungal Meningitis Outbreak which claimed 115 lives in Mexico. He highlighted one patient in particular who was admitted for caesarian section, following an uncomplicated pregnancy. She had an epidural anaesthetic but became very unwell, resulting in a staggering 2 years in hospital where she endured coma, two strokes and a 35 kilo weight loss. It took a long time to establish what had happened to her but it was discovered she had a fungal meningitis, *Fusarium Solani*.

Dr. Chejfec focused on the patient safety, traditional and contemporary. The traditional focused on errors and adverse events, the reactive approach (after harm occurs), seeking root cause (what went wrong) and individual accountability. The goal, to prevent failure.

The contemporary focuses on everyday clinical performance, a proactive approach (before harm occurs), seek understanding (how does care succeed) with emphasis on system interactions and adaptation. The goal, to ensure things go right.

The task force took a multidisciplinary collaboration with a shift from individual blame focusing on interactions, parallel investigation and system improvement.

It can come as quite a shock when you hear the reality of real life and the extent in which it has effect, and that not everyone is so fortunate as we are with the healthcare we have in the UK, and the things we take for granted. It was discovered that in Mexico there were huge supply gaps, medicines unavailable, limitations in regulations, unintended consequences like the emergence of black markets in order to get hold of drugs as there was no other way. Doctors would have to have their own 'tackle boxes' to carry supplies just to make sure they could actually have the drugs for these procedures as they may not have been available or risk being stolen. This meant drugs were subjected to being stored in hot cars in unacceptable temperature variance resulting in degradation of the medicines. Something unimaginable!

On top of this, there was inconsistency of PPE and limited access meaning asepsis was literally dependant on availability. All of these findings highlighted by the task force stated, 'safety is a system property, not created by an individual alone, not ensured by protocols alone and not maintained by vigilance alone. It is a collaboration and we are all responsible and accountable for safe care of our patients'.

The sheer power in this presentation was by telling the story of one individual and how it affected her life. She has said that she will never be the same but she is still with us and alive and can tell her story. A powerful and heart rendering presentation.

The Congress ran over three days and was packed with sessions covering many subjects including AI, Digitalised reasoning and decision making, the use of blocks and what to do if they fail, operating theatre and PACU design for the future to name just a few subjects included in day one. 124 posters were submitted and I was fortunate to be included in the 'Poster Round Up' sessions. That meant that I was able to listen to 16 poster presentations and meet the authors. It was a privilege to hear them and feel the enthusiasm and energy in these presentations and witness the innovation exhibited.

Early in the morning at 6.30am on day two there was a fun run which was really well attended with enthusiastic nurses, friends and colleagues setting off on a well organised run in the city of Brisbane leaving from the buzzing Southbank. I have to confess, I was not one of these amazing people!!

Day two covered the paediatric stream including obstetric anaesthesia, perioperative management of neonate and premature babies and other anaesthetic techniques. The IFNA Practice and Education Committees ran their streams which included preceptorship, competency and assessment. Workshops ran including double-lumen ET tube intubation, IV and PICC ultrasound guided insertion, front of neck airway management and advanced life support.

There was also the opportunity to hear about anaesthesia in urban vs rural areas in Rwanda, sedation outside of the operating theatre and areas of crisis management.

Day three, the last day of congress, had a stream focused on trauma. I was particularly looking forward to this as we heard about the day in the life of a nurse working with the Flying Doctors. Every day different, much like with our own air ambulance, but with the added dangers of Australia including many creatures that will cause you any amount of grief with their teeth, stings and bites, plus the inaccessible terrain of the outback, sheer distance and the hazards of everyday life! I felt great admiration for the work of this team and was stuck by their support for each other making for a very strong team.

I was interested to listen to the session about diving accidents and the hyperbaric chamber. I always look for subjects that may not be common place in any congress I may go to in the UK so that I can have a different perspective on practice elsewhere. This session, just like the Flying Doctors, did not disappoint. I had never appreciated the horrendous consequences if these chambers are not used or managed correctly. An absolute eye opener!

One of my roles as the Country Representative is to Chair some of the sessions. This is always a great privilege and a pleasure. I looked after presentations including Supplemental Oxygen Therapy, Postoperative Delirium and Fat Emboli. My speakers were from Croatia, Belgium and Finland.

It's a huge job putting together the World Congress which I can justify as I am speaking from experience! Currently we don't know where the next congress, WCNA 2030 will be held but as soon as I know, you will too as it will be advertised via the BARNA and IFNA

websites. We are hoping to run a regional congress inbetween as four years seems far away!

I can't tell you enough what a wonderful experience it is to attend the World Congress of Nurse Anaesthetists. Please do look out for the next one and consider coming along if you can. I couldn't possibly fit all the subjects in this report but suffice to say there was a huge amount to choose from. What I can tell you is that the atmosphere was electric, truly buzzing! To be with so many like minded people who are filled with enthusiasm and excitement for their profession is truly inspiring. It is also so interesting to discover we all have so much in common referring to our practice, problems and concerns. The support from the companies was rock solid and you could feel their enthusiasm for the delegates and their engagement with everyone which was great.

I would like to thank you for reading this report which I hope will have given you a little taste of the passion, fun and knowledge I experienced. Maybe see you at WCNA 2030.....!

With thanks and appreciation,

Manda Dunne

IFNA CNR (Council of National Representatives)

IFNA Executive Committee Member

BARNA Membership Secretary's Report 2026

Period covered: July 2025 to July 2026

I am pleased to present the Membership Secretary's Report for the period July 2025 to July 2026.

Our current membership stands at **90 members**, which includes **11 Board members**.

New Members

During the reporting year, we received **60 new membership applications**. Of these, **42 applicants completed the process and paid their membership fee**.

18 applicants did not complete payment and therefore did not become members.

Renewals

During the year, **37 members successfully renewed** their membership, demonstrating continued commitment to the Association.

Renewal reminders continue to be managed proactively. Members receive a reminder one month before their membership expires and a further reminder during their expiry month. A two-week grace period is then provided before membership and associated benefits are suspended. For example, a member whose membership expires at the end of June would retain access until the middle of July, allowing additional time to renew.

Trends

As in previous years, membership applications followed a seasonal pattern. The summer months remained our quietest period, while

activity increased significantly during the autumn and spring, particularly in the lead-up to the annual conference. The busiest months for new applications were **October, February and May**.

Incomplete Applications

The majority of the 18 incomplete applications may be a result of the current manual payment process.

As there is no instant payment facility when an application is submitted, some applicants may not return to complete payment after submitting their details.

Introducing an immediate online payment option at the point of application may improve conversion from application to paid membership.

I would like to thank the Board, especially Emma our Treasurer for her prompt communication regarding payments, without which my role would be impossible.

I look forward to continuing to help support our members both current and future, and the BARNA board to deliver educational and informative resources in the year ahead.

BARNA Strategic Plan 2026–2027

Vision

To be the leading voice for anaesthetic and recovery nursing, advancing excellence in perioperative care through education, professional development, advocacy and collaboration.

Mission

To support and represent recovery and anaesthetic practitioners by providing education, setting standards, promoting evidence-based practice and influencing healthcare policy to improve patient outcomes.

Strategic Priority 1:

Strengthen Professional Practice and Standards

Objective

Position BARNA as the recognised authority for recovery and anaesthetic nursing standards across the UK.

Key Priorities

Workforce Standards Programme

- Publish and implement the BARNA Safe Staffing and Workforce Planning Tool - **COMPLETE**
- Support organisations to benchmark their recovery services.
- Develop guidance on skill mix and competency frameworks.
- Create a national benchmarking network.

Clinical Guidance

- Update existing guidance documents.
- Collaborate with partner organisations on endorsement opportunities.
- Develop position statements on emerging issues affecting recovery services.

Quality Improvement

- Share examples of innovation and best practice.
- Establish annual awards recognising excellence in recovery practice.

Success Measures

- Workforce tool launched nationally.
- Benchmarking database established.
- At least two new professional guidance documents published.

Strategic Priority 2:

Deliver Outstanding Education

Objective

Become the primary source of specialist education for recovery and anaesthetic practitioners.

Key Priorities

National Conference Development

- Increase attendance year-on-year.
- Expand abstract and poster submissions.
- Introduce dedicated trainee and novice practitioner streams.

Education Portfolio

- Increase virtual learning opportunities including Podcast.
- Offer webinars throughout the year.

Career Development

- Create a recovery nursing career pathway resource.
- Support leadership development.
- Promote advanced practice opportunities.

Success Measures

- Year-round education calendar established.
- Increased conference attendance and abstract submissions.
- Positive delegate satisfaction scores.

Strategic Priority 3:

Grow Membership and Engagement

Objective

Build a sustainable and engaged membership community.

Key Priorities

Membership Growth

- Increase individual membership.
- Improve student and newly qualified nurse engagement.
- Develop benefits that clearly demonstrate membership value.

Community Building

- Develop regional representatives network.
- Encourage member participation in projects and working groups.
- Create opportunities for networking and peer support.

Volunteer Development

- Provide clearer role descriptions.
- Introduce trustee induction and development.
- Create succession plans for key leadership positions.

Success Measures

- Membership growth achieved year on year.
- Improved retention rates.
- Increased volunteer participation.

Strategic Priority 4:

Increase Influence and Advocacy

Objective

Ensure recovery and anaesthetic nursing has a strong voice in national healthcare conversations.

Key Priorities

National Representation

- Strengthen relationships with professional bodies.
- Participate in relevant consultations.
- Increase representation at national forums.

Policy and Advocacy

- Publish position statements on key professional issues.
- Advocate for safe staffing and patient safety.
- Promote recognition of recovery and anaesthetic nursing specialities.

International Collaboration

- Build relationships with international recovery nursing organisations.
- Share learning and best practice globally.

Success Measures

- BARNA representation on key national groups.
- Increased invitations to contribute to guidance and consultations.
- Growth in collaborative projects.

Strategic Priority 5:

Strengthen Governance and Sustainability

Objective

Ensure BARNA remains a well-governed, financially sustainable charity.

Key Priorities

Governance

- Review trustee structure and committee arrangements.
- Clarify trustee roles and expectations.
- Strengthen succession planning.

Financial Sustainability

- Diversify income sources.
- Develop sponsorship and partnership opportunities.
- Increase reserves in line with charity policy.

Communications and Brand

- Refresh website and digital presence.
- Improve communications strategy.
- Increase visibility of BARNA achievements and impact.

Success Measures

- Trustee succession plan in place.
- Financial targets achieved.
- Increased website and social media engagement.

Strategic Goals Dashboard

Goal	2026-27 Target
Membership growth	+15%
Conference attendance	+20%
National webinars	Minimum 6 annually
Workforce standards implementation sites	20+
New guidance documents	2–3
Podcast	4+
Trustee succession plans	100% of officer roles
Strategic partnerships	3–5 active partnerships